

MedEd-HalluScore v0.1

A Practical Framework for Evaluating Hallucination and Educational Safety Risks in LLM-Generated Clinical Cases

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Abstract

Large language models are increasingly capable of generating clinical cases for medical education. However, generated cases may contain factual errors, internal contradictions, missing safety-critical information, and misleading clinical reasoning. MedEd-HalluScore is a practical evaluation framework for scoring hallucination and educational safety risks in LLM-generated clinical cases. The framework uses six dimensions, each scored from 0 to 3, producing a total risk score from 0 to 18. This initial white paper describes the framework rationale, scoring method, intended use, limitations, and an initial validation plan.

1. Background

Clinical cases are central to medical education. They help learners practice diagnostic reasoning, identify relevant data, compare differential diagnoses, and connect clinical findings with management decisions. LLMs can accelerate case generation, but speed alone does not guarantee educational safety.

An AI-generated case may look fluent while containing incorrect facts, incoherent findings, omitted red flags, or unsafe teaching points. These failures are especially important in medical education because learners may not yet have enough expertise to recognize them.

2. Objective

MedEd-HalluScore aims to provide a structured, reproducible method for evaluating whether an LLM-generated clinical case is suitable for educational review and use.

The framework evaluates educational content. It is not designed for direct clinical care, diagnosis, treatment, triage, or patient-specific decision-making.

3. Framework Design

The framework includes six dimensions:

1. Medical factuality
2. Internal clinical consistency
3. Critical information omissions
4. Clinical reasoning risk
5. Educational safety
6. Transparency and verifiability

Each dimension receives a score from 0 to 3:

- 0: no apparent risk
- 1: minor risk
- 2: moderate risk
- 3: severe risk

The total score is the sum of all six dimensions.

4. Risk Classification

Total Score | Risk Level | Recommended Action

- 0-3 | Low Risk | Use after routine human review and minor edits.
- 4-8 | Moderate Risk | Use only after careful correction and qualified review.
- 9-13 | High Risk | Do not use without substantial revision and re-review.
- 14-18 | Critical Risk | Reject or fully rewrite before educational use.

5. Example Application

A chest pain case that describes crushing substernal pain radiating to the left arm but omits vital signs, ECG, and troponin, then recommends discharge with antibiotics, would likely score high or critical risk. The issue is not only factual error, but also the possibility of teaching an unsafe reasoning pattern.

6. Intended Users

Potential users include:

- Medical educators
- Clinical faculty
- Curriculum reviewers
- AI safety researchers

- Health education quality teams
- Students or trainees working under supervision

7. Initial Validation Plan

The initial validation plan includes feasibility testing, inter-rater reliability analysis, expert alignment, and external pilot use. Version 0.1.0 includes illustrative examples but should not yet be considered a validated benchmark.

8. Limitations

MedEd-HalluScore depends on reviewer expertise and context. It does not replace clinical judgment, institutional curriculum review, or specialty-specific validation. The framework should be revised as empirical evidence and external feedback accumulate.

9. Conclusion

MedEd-HalluScore offers a practical starting point for evaluating hallucination and educational safety risks in LLM-generated clinical cases. By combining a scoring rubric, sample dataset, reviewer templates, and a validation plan, it supports more transparent and accountable use of AI-generated material in medical education.